

Annex 4

Procedure for prequalification of pharmaceutical products

For any further information or request, please send an email to the Norms and Standards for Pharmaceuticals (NSP) Team at WHO, at email nsp@who.int.

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Abbreviations

API	active pharmaceutical ingredient
CRO	contract research organization
EOI	expression of interest
FPP	finished pharmaceutical product
GCP	good clinical practices
GLP	good laboratory practices
GMP	good manufacturing practices
HIV/AIDS	human immunodeficiency virus/acquired immunodeficiency syndrome
ML	maturity level
NRA	national regulatory authority
UNAIDS	Joint United Nations Programme on HIV/AIDS
UNFPA	United Nations Population Fund
UNICEF	United Nations Children's Fund
WHO	World Health Organization
WLA	WHO-listed authority

1. Introduction

The World Health Organization (WHO) provides United Nations agencies with advice on the acceptability, in principle, of pharmaceutical products for procurement by such agencies. This activity of WHO aims to facilitate access to priority essential medicines that meet WHO-recommended norms and standards of acceptable quality, safety and efficacy.

WHO undertakes a comprehensive evaluation of pharmaceutical products, based on information submitted by the manufacturers of such products or other applicants, and on an inspection of the corresponding manufacturing facilities and clinical sites. This is done through a standardized procedure that is based on WHO-recommended quality, safety and efficacy standards.

The pharmaceutical products found to meet the WHO-recommended quality, safety and efficacy standards are included in the list of prequalified finished pharmaceutical products, along with details of the specified manufacturing sites. These products are considered acceptable, in principle, for procurement

by United Nations agencies. The list of prequalified pharmaceutical products is principally intended for use by United Nations and other international agencies – including the Joint United Nations Programme on HIV/AIDS (UNAIDS), the United Nations Children's Fund (UNICEF), the United Nations Population Fund (UNFPA) and the Global Fund to Fight AIDS, Tuberculosis and Malaria – to guide their procurement decisions. The expanding list of pharmaceutical products that have been found to meet WHO-recommended standards may, however, also be of interest to other organizations and countries wishing to use the list to help inform their own regulatory decision-making and to engage in the bulk procurement of pharmaceutical products.

Inclusion in the list does not imply any approval by WHO of the pharmaceutical products and manufacturing sites in question, which is the sole prerogative of national regulatory authorities (NRAs). Moreover, inclusion in the list does not constitute an endorsement or warranty by WHO of the fitness of any product for a particular purpose, including its safety or efficacy in the treatment of specific diseases.

2. Glossary

The definitions given below apply to the terms used in this procedure. They may have different meanings in other contexts.

abridged prequalification assessment pathway. A regulatory pathway in which WHO considers prequalification of a product that has already been approved by a WHO-listed authority (WLA), fully relying on the WLA's prior evaluation and approval decision.

active pharmaceutical ingredient (API). Any substance or mixture of substances intended to be used in the manufacture of a pharmaceutical dosage form, and that, when so used, becomes an active ingredient of that pharmaceutical dosage form. Such substances are intended to furnish pharmacological activity or other direct effect in the diagnosis, cure, mitigation, treatment or prevention of disease, or to affect the structure and function of the body. It is also referred to as the drug substance.

applicant. The person or entity who, by the deadline mentioned in the invitation, submits an expression of interest to participate in this procedure in respect of the products listed in the invitation, together with the required documentation on such products.

comparator product. A pharmaceutical product with which the generic product is intended to be interchangeable in clinical practice. The comparator product

will normally be the innovator product for which efficacy, safety and quality have been established. For the Prequalification of Medicines Programme, the selection of the comparator product is based on the information presented under guidance on bioequivalence studies available on the prequalification website.

contract research organization (CRO). A commercial, academic or other type of organization to which an applicant may delegate certain tasks and responsibilities related to the conduct of clinical studies for a product submitted to WHO for assessment under the current procedure.

finished pharmaceutical product (FPP). A finished dosage form of a pharmaceutical product, which has undergone all stages of manufacture, including packaging in its final container and labelling. It is also referred to as the drug product.

full assessment prequalification pathway. A regulatory pathway in which WHO conducts a comprehensive evaluation of a product's quality, safety and efficacy, including inspections of manufacturing sites and contract research organizations. While assessment and inspection reports from WHO-listed authorities or maturity level 3 or 4 national regulatory authorities may be considered to facilitate the process, such prior approvals are not mandatory for this pathway.

innovator product. Generally, the pharmaceutical product that was first authorized for marketing (normally as a patented product) on the basis of documentation of efficacy, safety and quality.

invitation for expressions of interest (EOIs). Invitation calling upon interested parties (such as manufacturers or other applicants) to submit an expression of interest (EOI) to WHO by a specified deadline for the purpose of participating in the WHO prequalification procedure in respect of the products listed in the invitation. Such an EOI should be accompanied by the required documentation on the products in question.

manufacturer. A company that produces, packages, repackages, labels or relabels pharmaceutical products.

maturity level 3 or 4 national regulatory authority (ML3/ML4 NRA). A national regulatory authority or a regional regulatory system included in the WHO list of national regulatory authorities operating at maturity level 3 or 4, as benchmarked against the Global Benchmarking Tool.

Only in relation to good manufacturing practice inspections: a medicine regulatory authority that is a member of the Pharmaceutical Inspection Co-operation Scheme, as specified at <http://www.picscheme.org>.

multisource (generic) pharmaceutical product. A pharmaceutically equivalent or pharmaceutically alternative product that may or may not be therapeutically equivalent. Multisource pharmaceutical products that are therapeutically equivalent are interchangeable.

pharmaceutical product. Any substance or combination of substances marketed or manufactured to be marketed for treating or preventing disease in human beings, or with a view to making a medical diagnosis in human beings, or to restoring, correcting or modifying physiological functions in human beings.

prequalification. Standardized safety, efficacy and quality assessment procedure of WHO to evaluate the acceptability, in principle, of pharmaceutical products for purchase by United Nations agencies. Prequalification also supports national regulatory authorities in undertaking facilitated national regulatory decisions and procurement agencies in identifying quality-assured sources of essential medicines. Agencies using information resulting from the prequalification procedure should perform additional steps of qualification prior to purchasing, such as ensuring financial stability and standing of the supplier, ability to supply the required quantities, security of the supply chain, preshipment quality control and other related aspects.

WHO-listed authority (WLA). A national regulatory authority or a regional regulatory system that has been documented to comply with all the relevant indicators and requirements specified by WHO for the requested scope of listing based on an established benchmarking and performance evaluation process. A regulatory authority is meant to cover all the institutions, working together in an integrated and effective manner, that are responsible for regulatory oversight of medical products in a given country or region.

3. Purpose and principles

The purpose of this WHO procedure is to evaluate whether certain pharmaceutical products (considered by WHO to be vital for the prevention and treatment of diseases, including human immunodeficiency virus/acquired immunodeficiency syndrome (HIV/AIDS), tuberculosis, malaria and other diseases, or for reproductive health) meet the requirements recommended by WHO (as published on the WHO prequalification website) and are manufactured in compliance with current good manufacturing practices (GMP).

To respond to the changing needs of global health, the procedure may be expanded to include noncommunicable diseases, such as cancer, cardiovascular diseases and diabetes. The growing dependence on biotherapeutic and biosimilar products for treating both infectious and non-infectious conditions further

highlights the importance of expanding the scope of prequalification. This ensures equitable access to quality-assured health products across a wide range of therapeutic areas.

This procedure established by WHO is based on the following principles:²

- listing of the medicines eligible for prequalification in invitations for expressions of interest (EOIs) published on the WHO website;
- a general understanding of the production and quality control activities of the manufacturer;
- assessment of pharmaceutical product data (information related to quality, safety and efficacy), as submitted by the manufacturer;
- Inspection of finished pharmaceutical product (FPP) and active pharmaceutical ingredient (API) manufacturing sites for compliance with GMP;
- inspection of clinical testing units or contract research organizations (CROs) performing clinical trials (including bioequivalence studies) for compliance with current good clinical practices (GCP), current good laboratory practices (GLP) and other applicable guidelines;
- reliance on the information supplied by WHO-listed authorities (WLAs) and maturity level 3 or 4 (ML3/ML4) national regulatory authorities (NRAs) (as appropriate and as qualified later in this document and in the associated guidance documents);
- random sampling and testing of supplied pharmaceutical products;
- handling of complaints, product recalls and other major quality incidents (for example, out-of-specification events experienced during stability studies) reported to WHO;
- monitoring of complaints received from agencies and countries.

WHO may collaborate with NRAs in conducting dossier assessments and inspections. Subject to the terms of section 4 below, the prequalification of a product may also be based, fully or in part, on approval by a WLA or an ML3/ML4 NRA.

Given that many countries still operate under national regulatory systems at early stages of maturity (for example, maturity levels 1 and 2), WHO recognizes the importance of including enabling mechanisms for such contexts. This includes provision of trainings of various forms and participation of NRA staff in prequalification assessment and inspection activities.

WHO recommends that applicants expressing interest in participation in the prequalification procedure inform the NRAs in the country of manufacture of their intention and request them to collaborate with WHO in the WHO product assessment process. It is recommended that applicants provide the

NRAs with the necessary authorization to discuss the relevant product files with WHO representatives during dossier assessment and site inspections (subject to appropriate confidentiality provisions, if necessary). This collaboration facilitates information sharing and strengthens the overall effectiveness of the prequalification process.

WHO also encourages structured collaboration with the NRAs in both manufacturing and procuring countries. This could include regular feedback loops, participation in dossier review sessions, and joint inspections where appropriate. These collaborations foster regulatory capacity-building and support harmonization efforts at the regional and global levels.

4. Steps of the procedure

WHO undertakes a comprehensive evaluation of the quality, safety and efficacy of pharmaceutical products, based on information submitted by the applicants and inspection of the relevant manufacturing and clinical or bioequivalence study sites. Flowcharts showing the prequalification process for the various pathways are provided in Appendices 1–3.

This evaluation is conducted in accordance with WHO-recommended quality standards and includes a thorough review of the product dossier, assessment of manufacturing practices and verification of clinical data, where applicable. The process aims to ensure that only pharmaceutical products meeting international standards of quality, safety and efficacy are eligible for procurement consideration.

Taking into consideration pertinent input received from Member States, the United Nations and other agencies, WHO will publish an invitation to interested parties, inviting them to voluntarily participate in this procedure in respect of the products specified in the invitation. Invitations are updated from time to time to reflect current medical and public health needs.

By submitting an EOI, the applicant commits to sharing comprehensive information with WHO on all relevant aspects of the quality, safety and efficacy of the specified product, including any changes made or planned. The interested applicant must provide the required information to WHO by submitting a product dossier in the prescribed format, along with any additional documentation, as requested.

The procedure will normally include:

- assessment of product dossiers, which must include all required product data and information, as outlined in the submission guidelines available on the WHO website;
- inspection of manufacturing sites of FPPs and APIs, to assess compliance with GMP;

- inspection of clinical or bioequivalence sites (if applicable), to assess compliance with GCP and GLP, as appropriate.

This information forms the basis for WHO's comprehensive evaluation of product quality, safety and compliance with applicable regulatory standards. If the evaluation demonstrates that a product and its corresponding manufacturing (and clinical or bioequivalence) sites meet WHO-recommended standards, the product will be included in the list of pharmaceutical products that are considered to be acceptable, in principle, for procurement by United Nations and other international agencies (that is, the product is considered to be "prequalified").

WHO reserves the right to terminate the evaluation of a specific product if the applicant is not able to provide the required information, or is unable to implement any corrective actions that WHO may require within a specified time period, or when the information supplied is inadequate to complete this procedure. Similarly, the applicant may request the termination of the product evaluation by submitting a formal notice of withdrawal.

WHO may utilize relevant reliance mechanisms for prequalification of products approved by WLAs or ML3/ML4 NRAs or regional regulatory systems that apply standards for quality, safety and efficacy equivalent to those recommended by WHO. Provided that these authorities are willing to share certain information with WHO, such products may be considered for inclusion in the list of WHO-prequalified products through either an abridged or a facilitated or reliance-based full assessment pathway, as highlighted below and outlined in the applicable submission guidelines on the WHO prequalification website. Inclusion in the WHO-prequalified list is contingent upon the applicant's expressed interest in pursuing this pathway and the availability of the required information.

For products approved by WLAs, the abridged pathway applies to all invited products and relies on the scientific assessments and inspections conducted by the WLA, along with the exchange of relevant information. WHO will consider such products for prequalification when the prequalification applicant expresses interest in prequalification via this pathway and when the necessary information becomes available.

For products approved by ML3/ML4 NRAs or regional regulatory systems, the facilitated or reliance-based full assessment pathway⁵ applies only to eligible products or product types identified in the associated guidance available on the WHO prequalification website. WHO will consider these products based on the complete dossier submitted by the applicant, the assessments and inspections conducted by the NRA, and the exchange of relevant information.

⁵ Such reliance on the work of the ML3/ML4 NRA could be partial, for example with respect to the quality assessment only or the safety or efficacy assessment only, or complete, covering all aspects of the dossier.

A WHO inspection of a manufacturer or CRO may not be required if all the following conditions are met:

- there has been an inspection by a WLA;
- the inspection was conducted within the last three years;
- information on the inspection (including inspection report and responses to any deficiencies) is available for review by WHO;
- based on this and other available information, it is determined⁶ that the sites in question meet the applicable WHO-recommended standards.

WHO may utilize inspection reports issued by Pharmaceutical Inspection Co-operation Scheme member inspectorates or ML3/ML4 NRAs as part of a desk review of available inspection information on manufacturing sites and clinical or bioequivalence studies.

With a view to coordinating inspection activities, avoiding duplication and promoting information sharing without prejudice to the protection of any confidential or proprietary information of the applicants and manufacturers in accordance with the terms of this procedure, WHO may disclose inspection-related information to regulatory authorities of WHO Member States as well as to regulatory authorities that are members of the Pharmaceutical Inspection Co-operation Scheme. By participating in the prequalification process, the applicant agrees to information-sharing activities between regulatory authorities within the scope outlined.

5. Invitation for expressions of interest

The pharmaceutical products listed in an invitation for EOIs are considered by WHO to be vital for the effective treatment and prevention of the specified diseases (for example, HIV/AIDS, malaria, tuberculosis, and others) or for reproductive health. These products are normally included in either the WHO Model List of Essential Medicines or the relevant WHO treatment guidelines and recommendations (or both).

The products included in the WHO Model List of Essential Medicines are those that satisfy the priority health care needs of a population. They are selected, among other criteria, on the basis of disease prevalence, evidence on efficacy and safety, and analysis of comparative cost-effectiveness. Products included in WHO

⁶ Taking into account any known specific risks associated with the products, the results of previous inspections conducted by WHO, a WLA, a Pharmaceutical Inspection Co-operation Scheme inspectorate or an ML3/ML4 NRA, any complaints (if known), the scope and detail of the inspection report, the number and type of any GMP deficiencies reported, the comprehensiveness of the manufacturer's response and the timelines for implementation of corrective actions.

treatment guidelines are selected on the basis of an assessment of the evidence for benefits, risks, costs and appropriateness for use in a variety of situations, taking into account the needs of special populations and the values and preferences of the groups (professional and patient) using them.

To remain responsive to evolving global and regional health priorities, WHO may include product categories or therapeutic areas not yet covered. Such inclusion would be guided by public health need, disease burden and alignment with WHO treatment guidelines or policy recommendations.

Each invitation will be open and transparent, inviting all relevant parties to submit an EOI for the pharmaceutical products listed. Such invitations are typically published on the WHO website and may also be disseminated through other channels.

In situations of high public health concern as determined by WHO, the Organization may also directly invite relevant parties to submit specified product dossiers for evaluation by WHO under this procedure without publication of an invitation for an EOI.

6. Data and information to be submitted

Interested parties are expected to submit documentation on the pharmaceutical products as called for in the invitation for EOIs. Applicants should submit their product dossiers with the required information to the WHO focal point while the product remains invited. Guidance and instructions developed for the submission of the dossiers are made available on the WHO website.

Normally, applicants participating in the WHO prequalification procedure for pharmaceutical products are the manufacturers of the FPPs, as specified in the invitations for EOIs. If an applicant is not the manufacturer of the FPP, all relevant documentation, including contract manufacturing agreements, must be submitted to demonstrate that the applicant has complete knowledge of and is accountable for the manufacturing process and quality assurance of the products submitted for prequalification.

In submitting an EOI for product evaluation, the applicant should send the following to the WHO focal point:

- a covering letter, expressing interest in participating in the WHO prequalification procedure and confirming that the information submitted in the product dossier is complete and correct;
- a product dossier, in the format specified in the WHO guidance documents on submitting product data and information;
- product samples, for purposes of visual examination; WHO may also ask for product samples for chemical and pharmaceutical analysis, if deemed necessary;

- a site master file for each manufacturing site listed in the product dossier, in the format specified in the WHO guidance documents for submitting a site master file;
- a CRO master file for each clinical site listed in the dossier, in the format specified in the WHO guidance documents for submitting a CRO master file.

All documentation should be submitted in English.

For the purposes of this procedure, different requirements for documentation to be submitted apply to the following categories of products:

- multisource (generic) FPPs to be assessed by WHO;
- multisource (generic) FPPs approved by ML3/ML4 NRAs;
- innovator or multisource (generic) FPPs approved by WLAs.

The documentation requirements for each of the above categories are available on the WHO prequalification website and may be updated periodically.

Multisource generic products must be demonstrated, either directly or indirectly, to be therapeutically equivalent to the comparator product to be considered interchangeable. WHO maintains and publicly shares the list of comparator products for this purpose. The WHO website provides guidance on the evidence required for a product to be considered equivalent without the need for in vivo bioequivalence studies (that is, application of a biowaiver).

If considered necessary or desirable by either party, and before the actual evaluation process starts, a discussion may be held between the manufacturer and WHO. This meeting should be scheduled as early as possible, with a predefined agenda to address questions sent in advance to WHO by the manufacturer.

7. Screening of dossiers submitted

Each product dossier submitted by an applicant will be screened for completeness before being evaluated according to the screening checklist found on the WHO prequalification website. Dossiers submitted for products that are not listed in an invitation for EOIs or have not otherwise been invited by WHO will not be accepted for assessment.

Similarly, WHO will not consider dossiers that are incomplete. The applicant will be informed that an incomplete dossier has been received and will be requested to complete the dossier within a specified time period. In the event of non-compliance, the dossier may be rejected on grounds of incompleteness. Dossiers that are considered complete as the result of the screening will be retained by WHO for assessment.

After screening, if the dossier is accepted for assessment the applicant will be informed of this, including the dossier reference number, by letter. This letter will serve as an agreement between WHO and the applicant for the participation in prequalification and a commitment to comply with the provisions of the prequalification procedure. The WHO performance metric/key performance indicator clock will start on the date of the acceptance letter.

8. Dossier assessment

The product information submitted in the dossiers will be assessed by teams of experts (assessors) appointed by WHO. The assessors involved in dossier assessment must have the relevant qualifications and experience in the fields of pharmaceutical development, quality assessment of pharmaceutical products, quality assurance, biopharmaceutics and other relevant fields. The assessors will be appointed in accordance with a standard operating procedure established by WHO. The assessors should preferably be from NRAs or have worked at NRAs, and they will act as temporary advisers to WHO. The assessors must comply with the confidentiality and conflict of interest rules of WHO, as laid down in the relevant sections of this procedure.

The assessment of product dossiers will be done in accordance with standard operating procedures established by WHO for that purpose so as to ensure uniformity in evaluation and timeliness of assessment activities. If needed, WHO may provide training to these experts.

Each applicant may request a hearing or meeting with the WHO experts involved in the assessment of this applicant's dossier to clarify issues identified by the WHO experts. WHO (but not the prequalification teams) may provide technical assistance to applicants regarding appropriate product information to be submitted as well as production and control requirements.

9. Site inspection

WHO will plan and coordinate, in accordance with standard operating procedures established by WHO and based on quality risk management principles, the performance of inspections of the sites of manufacture of the APIs and the FPP, and of the clinical testing units or CROs.

The following factors will be taken into account when planning inspections:

- the results of previous inspections of the concerned sites by WHO, WLA, ML3/ML4 NRA or Pharmaceutical Inspection Co-operation Scheme member inspectorate, and history of compliance of the company or facility with GMP, GCP or GLP, as appropriate;

- the outcome of the assessment of data submitted to WHO;
- complexity of the site, processes and product;
- number and significance of known quality defects (such as complaints or recalls);
- major changes to buildings, equipment, processes or key personnel since the previous inspection;
- site experience with manufacturing and testing of a product;
- test results of official control laboratories.

The inspections of the manufacturing sites are conducted to assess compliance with GMP, as recommended by WHO, and include data verification. Site master files submitted by the applicant will be reviewed before an inspection is performed.

The inspections of clinical testing units or CROs are carried out to assess compliance with GCP and GLP, and to perform data verification.

The WHO norms and standards applicable to inspections of APIs and FPPs, and of clinical testing units or CROs, can be found on the WHO website. These requirements may be revised from time to time.

The inspections will be performed by a team of inspectors usually including experts appointed by WHO, preferably from NRA inspectorates, who will act as temporary advisers to WHO. The inspectors must have the relevant qualifications and experience to perform such inspections, be competent in areas such as production and quality control of pharmaceuticals, and have appropriate experience in GMP and GCP or GLP. The inspectors must comply with the confidentiality and conflict of interest rules of WHO, as laid down in the relevant sections of this procedure. If needed, WHO may provide training to these experts.

A WHO staff member will coordinate the team and will normally lead the inspection team. Each team will perform the inspections and report its findings to WHO in accordance with standard operating procedures established by WHO for that purpose, so as to ensure a standard harmonized approach. A representative of the NRA of the country of manufacture would normally be expected to accompany the team to the manufacturing and testing facilities to help assess the compliance with GMP and GCP or GLP.

In accordance with standard operating procedures established by WHO and based on quality risk management principles, an on-site inspection by a WHO inspection team may be waived provided that the site in question is found to meet the applicable WHO-recommended standards following a desk review of requested inspection reports, the manufacturer's responses to the relevant inspectorate describing corrective actions to any deficiencies identified in the

inspection reports, and other documents requested by WHO, including an acceptable product quality review report for the identified products.⁷

10. Reporting and communication of the results of the evaluation

Each assessment and inspection team will finalize its reports according to the established WHO standard operating procedure and format, describing the findings and including recommendations to the applicant, manufacturer or CRO, where relevant.

The findings from the dossier assessment, including deficiencies of the documentation and data submitted, shall be communicated in writing to the applicant requesting submission of the missing data and information, as appropriate, within a specified time frame. Applicants are expected to respond promptly to comments and provide any additional information requested. Responses addressing all queries should be submitted to WHO within the specified time frame indicated in each deficiency letter. The procedure is typically suspended (that is, WHO will not take further action) until all required responses and additional information have been received. If the deficiencies outlined in the letter are not responded to within the assigned time frame, the application will be terminated (cancelled). It will need to be resubmitted at a later date with all of the requisite information as a new application and with the associated fees.

The inspection report will be communicated to the manufacturer or CRO, as applicable. With the written agreement of the manufacturer or CRO, a copy of the inspection report may also be provided to the applicant (if other than the manufacturer or CRO). If any additional information is required, or corrective action has to be taken by the manufacturer or CRO, WHO will postpone its decision on the acceptability of the sites concerned until such information has been evaluated or the corrective action has been taken and found satisfactory in light of the specified standards. If these data are not submitted in the requisite time frame, the application will be terminated (cancelled). It will need to be resubmitted as a new application at a later date with the requisite data and all applicable fees.

WHO will terminate this procedure for a specific product if the applicant is not able to provide the required information or implement the corrective actions within a specified time period, or if the information supplied is inadequate to complete this procedure. In addition, each application may undergo only a limited number of additional data assessment rounds, as defined in Appendices 1–3 of

⁷ An inspection of a manufacturer or CRO may not be required if conditions described in section 4 are met.

this guideline or indicated on the WHO prequalification website. If, by the end of the final specified round, the application is still considered deficient, it will be terminated (cancelled). Similarly, the applicant reserves the right to ask WHO to terminate the procedure for a given application for any reason with a written letter of withdrawal. In any case, to pursue prequalification again, a new application must be submitted at a later date, along with the required data and applicable fee.

In the event of any disagreement between an applicant and WHO, a standard operating procedure established by WHO for the handling of such disagreements will be followed to discuss and resolve the issue.

As WHO is responsible for the prequalification procedure, the ownership of the reports lies with WHO. Thus, WHO shall be entitled to use and publish such reports, always, however, subject to the protection of any commercially sensitive confidential information of the applicant, manufacturer or testing organization. “Confidential information” in this context means:

- confidential intellectual property, know-how, industrial and trade secrets (including formulas, processes or information contained or embodied in a product, unpublished aspects of trade marks or patents);
- commercial confidences (such as structures and development plans of a company).

Provisions of confidentiality will be contained in the exchange of letters, to be concluded before the assessment of the product dossier or inspection of the manufacturing and clinical sites, between WHO and each applicant, manufacturer or CRO.

Notwithstanding the foregoing, WHO reserves the right to share the full assessment and inspection reports with the relevant authorities of any interested Member State of the Organization and interested United Nations agencies.

11. Outcome of the prequalification procedure

Once WHO is satisfied that this procedure is complete for the relevant product, and that the WHO-recommended standards are met, the product, as manufactured at the specified manufacturing site or sites, will be included in the list of prequalified pharmaceutical products. The list of prequalified pharmaceutical products will be compiled in accordance with a standard operating procedure established by WHO for final decision-making on inclusion in the list. The list will be published on the WHO website and will specify the characteristics of the prequalified pharmaceutical products, as described in Appendix 4 to this procedure.

Each applicant will receive a letter of prequalification from WHO informing it of the outcome of the prequalification dossier assessment process

in regard to the submitted products. Once the products are included in the list of prequalified pharmaceutical products, the applicant shall be responsible for keeping WHO continuously updated on all relevant aspects of the manufacture and control of such products and meeting any requirements, as agreed with WHO.

In accordance with World Health Assembly resolution WHA57.14 of 22 May 2004, WHO will – subject to the protection of any commercially sensitive confidential information – publish WHO public assessment reports on the product dossier assessments and WHO public inspection reports on the manufacturers and CROs that were found to be in compliance with WHO-recommended guidelines and standards. These reports will be published on the WHO website. Subject always to the protection of commercially sensitive confidential information, WHO shall also be entitled to publish negative evaluation outcomes in accordance with standard operating procedures established by WHO. These include notices of concern as well as notices of suspension.

The decision to list a pharmaceutical product is made based upon information available to WHO at that time, that is, information in the submitted dossier and on the status of GMP, GLP and GCP at the facilities used in the manufacture and testing of the product at the time of the site inspection conducted by WHO, a WLA, a Pharmaceutical Inspection Co-operation Scheme member inspectorate or an ML3/ML4 NRA, the outcome of which has been determined by WHO to meet the applicable WHO-recommended standards, in accordance with the terms of this procedure. This decision is subject to change on the basis of new information that may become available to WHO.

12. Maintenance of prequalification status

Applicants are required to communicate details to WHO of any changes (variations) in manufacture and control that may have an impact on the safety, efficacy and quality of the product.

Guidance on variations to prequalified dossiers can be found on the WHO website. These requirements may be revised from time to time.

It is the applicant's responsibility to provide WHO with the appropriate documentation (referring to relevant parts of the dossier) to prove that any intended or implemented variation will not have a negative impact on the quality, safety and efficacy of the product that has been prequalified. WHO will undertake an evaluation of variations according to the established WHO guidelines and standard operating procedures and communicate the outcome to the applicant within the prescribed timeline. Adherence to the reporting requirements will be verified during the inspections carried out by WHO.

For products approved via the facilitated or reliance-based full assessment procedure (underlying approval by an ML3/ML4 NRA), available variation

assessment reports issued by the ML3/ML4 NRA may be used to support WHO's assessment.

Changes or variations to products prequalified via the abridged procedure should be submitted to and approved by the WLA. WHO should then be notified of the approved changes according to the guidance published on the WHO prequalification website.

Random samples of prequalified products supplied by listed manufacturers or applicants may be taken for independent testing of final product characteristics. Certificates of analysis of final products released by the manufacturer and specifications for test methods should be provided by the manufacturer or applicant to WHO for review upon request. In the event of failure to meet the established criteria for testing, WHO will investigate the problem and communicate the outcome of this investigation to the manufacturer and applicant, if other than the manufacturer.

If safety, efficacy or quality concerns arise in relation to a prequalified product, WHO may delist the product after evaluation of the new evidence and a risk-benefit assessment, or may suspend the product until results of further investigations become available and are evaluated by WHO.

Complaints concerning prequalified pharmaceutical products communicated to WHO will be investigated in accordance with a standard operating procedure established by WHO for that purpose. After investigation, WHO will provide a written report of the problem and include recommendations for action where relevant. WHO will make the report available to the applicant or manufacturer, and to the NRA of the country where the manufacturing site is located. Subject always to the protection of commercially sensitive information as referred to above, WHO shall be entitled to make such reports public. In addition, WHO reserves the right to share the full report with the relevant authorities of interested Member States of the Organization and interested United Nations agencies.

Manufacturers of prequalified pharmaceutical products and associated API manufacturers will be reinspected at regular intervals as determined by WHO, but normally at least once every three years. Reinspection is conducted to verify compliance with GMP, as recommended by WHO, and includes data verification. Reinspection may also be conducted as part of investigating product quality complaints.

For products prequalified via the abridged procedure (underlying approval by a WLA), verification of inspection status of the manufacturing sites, or (if needed) desk review of inspection reports or an on-site inspection, may be undertaken as part of a requalification procedure described below, or earlier if needed.

Furthermore, in order to maintain their prequalification status, WHO will arrange for prequalified pharmaceutical products to be requalified at regular intervals.

Every five years from the date of prequalification, or when requested to do so by the WHO Prequalification of Medicines Programme, the holder of a prequalified product is required to submit data and information in relation to the product to WHO for assessment. The purpose of this assessment is to verify that the product conforms to information and data submitted in relation to prequalification and conforms to current norms and standards, and to verify the consistency of the quality of the product and its manufacturing processes over the identified period.

The procedure and guidelines on the requalification of prequalified products can be found on the WHO website. These requirements may be revised from time to time. Reinspection or requalification may also be performed:

- if any fraud or omissions by the applicant, manufacturer of an FPP or API, or CRO in the initial assessment procedure or during the follow-up activities become evident;
- if WHO, following investigation of product quality complaints or similar concerns, considers that a batch or batches of supplied prequalified pharmaceutical products are not in compliance with the specifications that were found to be applicable upon prequalification.

If, as a result of reinspection or requalification, it is found that a product or specified manufacturing site no longer complies with the WHO-recommended standards, such products and manufacturing sites may be suspended or removed from the list of prequalified pharmaceutical products. Failure of a manufacturer or applicant to participate in reinspection or requalification (as applicable) may also lead to suspension or removal from this list.

The applicant may request WHO to delist a given product for any reason based on a written request indicating the reason for withdrawal.

13. Cost recovery

WHO reserves the right to charge for this procedure on a cost recovery basis. Applicable fees are published on the WHO prequalification website.

14. Confidentiality undertaking

The assessors and inspectors will treat all information to which they will gain access during the assessments and inspections, or otherwise in connection with the discharge of their responsibilities in regard to the above-mentioned project, as confidential and proprietary to WHO or parties collaborating with WHO in accordance with the terms set forth below.

Assessors and inspectors will take all reasonable measures to ensure that confidential information:

- is not used for any purpose other than the assessment or inspection activities described in this document;
- is not disclosed or provided to any person who is not bound by similar obligations of confidentiality and non-use as contained herein.

Assessors and inspectors will not, however, be bound by any obligations of confidentiality and non-use to the extent they are clearly able to demonstrate that any part of the confidential information:

- was known to them prior to any disclosure by or on behalf of WHO (including by manufacturers); or
- was in the public domain at the time of disclosure by or on behalf of WHO (including by manufacturers); or
- has become part of the public domain through no fault of theirs; or
- has become available to them from a third party not in breach of any legal obligations of confidentiality.

15. Conflict of interest

Before undertaking the work, each assessor and inspector will also (in addition to the above-mentioned confidentiality undertaking) be required to sign a declaration of interest. If, based on this declaration of interest, it is felt that there is no risk of a real or perceived conflict of interest (or it is felt that there is only an insignificant or irrelevant conflict of interest), and it is thus deemed appropriate for the assessor or inspector in question to undertake this work, they will discharge their functions exclusively as adviser to WHO. In this connection, each assessor and inspector is required to confirm that the information disclosed by them in the declaration of interest is correct and complete, and that they will immediately notify WHO of any change in this information.

All inspectors furthermore agree that, at the manufacturer's or CRO's request, WHO will advise the manufacturer or CRO, in advance, of the identity of each inspector and the composition of the team performing the site inspection, and provide curricula vitae of the inspectors. The manufacturer or CRO then has the opportunity to express possible concerns regarding any of the inspectors to WHO before the visit. If such concerns cannot be resolved in consultation with WHO, the manufacturer or CRO may object to a team member's participation in the site visit. Such an objection must be made known to WHO by the manufacturer

or CRO within 10 days of receipt of the proposed team composition. In the event that such an objection is found to be meritorious, WHO reserves the right to cancel all or part of its agreement with, and the activities to be undertaken by, that inspector.

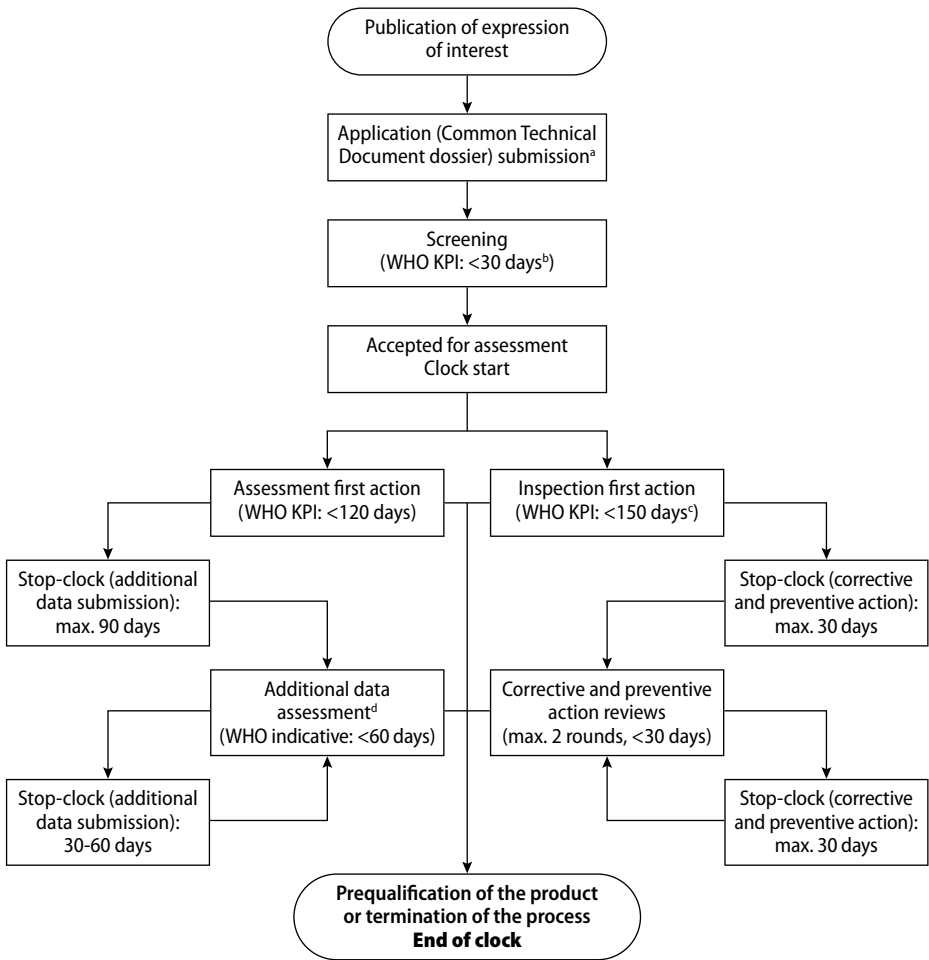
16. Digital tools and submission platforms

To enhance efficiency and facilitate participation, WHO continues to adopt digital tools throughout the prequalification process. This includes the use of secure e-submission platforms, remote inspection capabilities using digital technologies, real-time dossier tracking dashboards and automated workflows. These tools aim to streamline communication, facilitate processing of applications and reduce processing times.

Appendix 1. Full assessment pathway for multisource products

[Figure A4.1](#) illustrates the end-to-end assessment pathway for multisource pharmaceutical products, from submission of the Common Technical Document (CTD) dossier through screening, assessment, inspection, and final decision. Key WHO performance targets are indicated at each stage, including screening (<30 days), assessment (<120 days), inspection (<150 days), and additional data review (<60 days). Stop-clock periods apply for applicant responses, including corrective and preventive actions. Total WHO assessment time is targeted at less than 270 calendar days under standard conditions, excluding reinspection or new bioequivalence data requirements.

Fig. A4.1
High-level process flow and key performance targets: full assessment pathway for multisource products



Notes:

KPI: key performance indicator.

^a Requirements are described in separate guidelines.

^b All WHO and stop clock times are in calendar days.

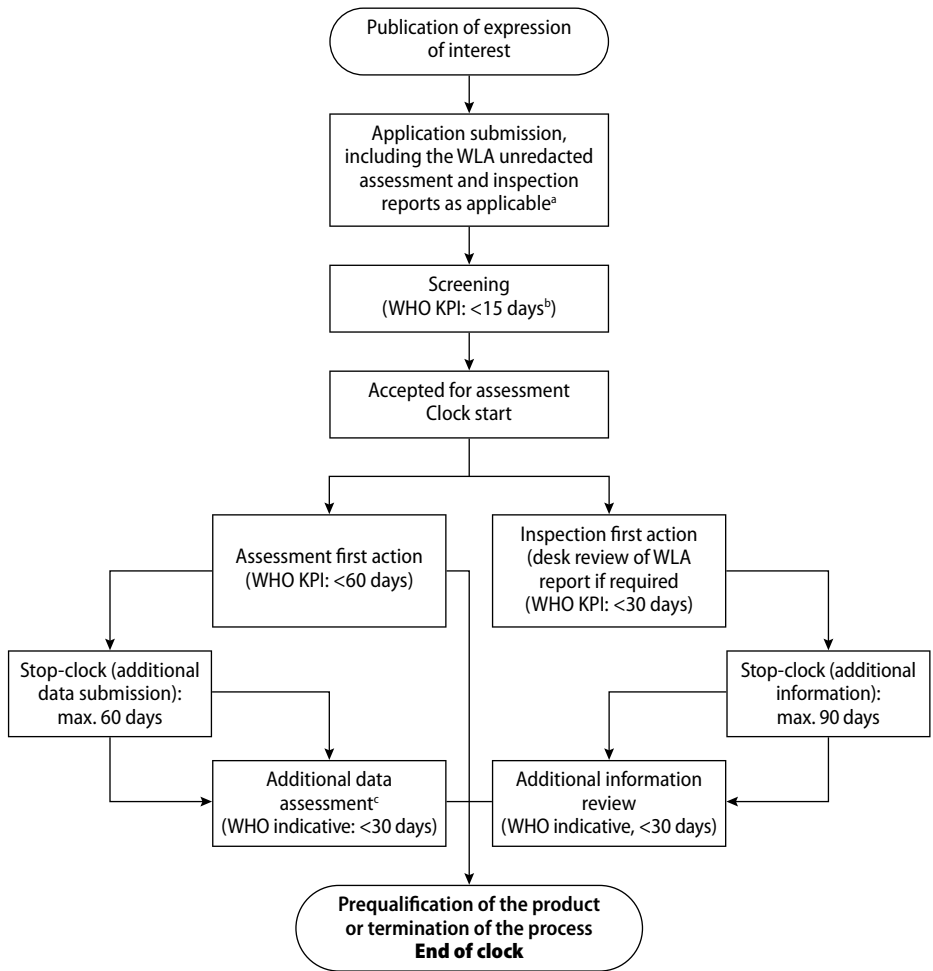
^c Assuming all sites are ready for inspection when the dossier is accepted for assessment. Less than 120 days if on-site inspection is not required.

^d Only limited rounds of additional data submission and assessment are possible.

Appendix 2. Abridged assessment pathway for multisource or innovator products approved by WHO-listed authority (WLA)

[Figure A4.2](#) illustrates the end-to-end abridged assessment pathway for multisource or innovator pharmaceutical products approved by WHO-listed authority (WLA). Key WHO performance targets are indicated at each stage, including screening (<15 days), assessment (<60 days), inspection (<30 days), and additional data review (<30 days). Stop-clock periods apply for additional data requests. Total WHO assessment time is targeted at less than 90 calendar days.

Fig.A4.2
High-level process flow and performance targets: abridged assessment pathway for multisource or innovator products approved by WLA

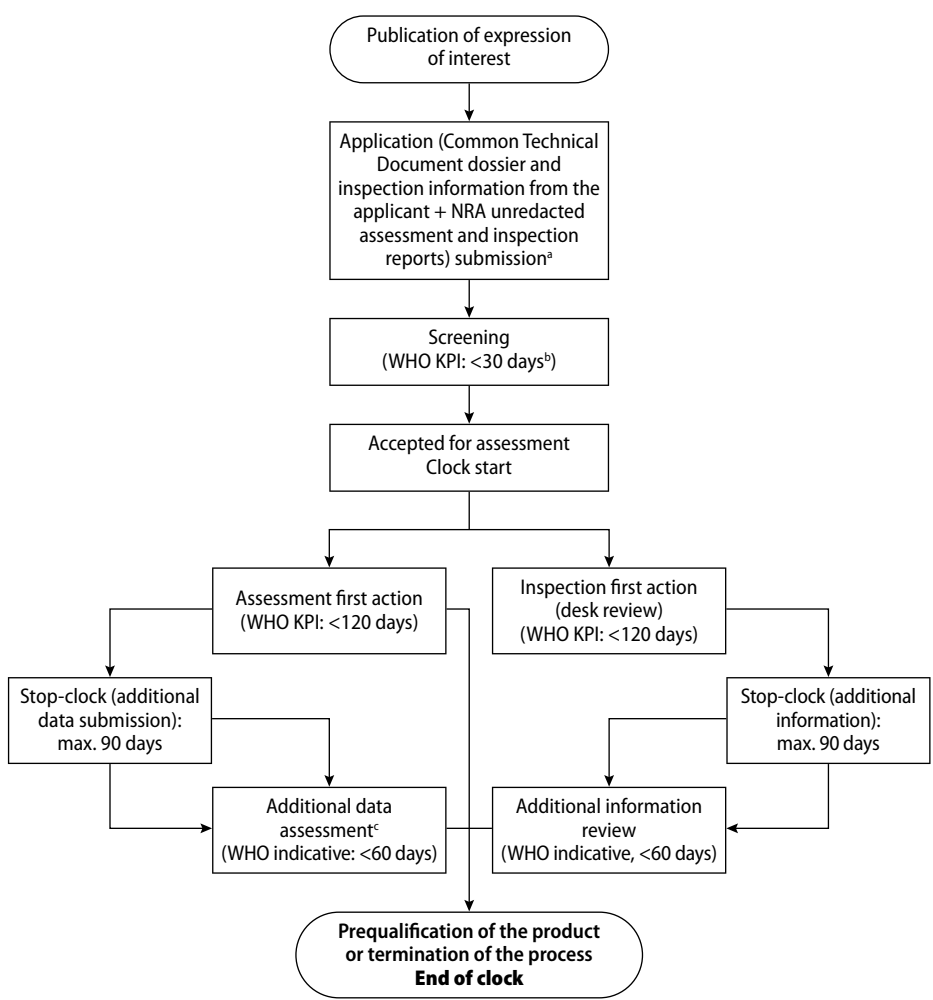


Notes:
 KPI: key performance indicator.
 WLA: WHO-listed authority
^a Requirements are described in a separate guideline.
^b All WHO and stop clock times are in calendar days.
^c If WHO cannot prequalify the product by the end of the second round, the application process will be terminated (i.e., the application will be cancelled). The application may also be cancelled if WHO determines that an on-site inspection of the manufacturing sites or the contract research organization is necessary after reviewing the available information.

Appendix 3. Full assessment pathway for multisource products facilitated by access to ML3/ML4 national regulatory authority or regional regulatory system reports

[Figure A4.3](#) illustrates the end-to-end assessment pathway for multisource pharmaceutical products facilitated by access to inspection and assessment reports from maturity level 3 or 4 national regulatory authority or regional regulatory system reports. Key WHO performance targets are indicated at each stage, including screening (<30 days), assessment (<120 days), inspection (<120 days), and additional data review (<60 days). Stop-clock periods apply for additional information submission. Total WHO assessment time is targeted at less than 180 calendar days when all conditions are met.

Fig.A4.3
High-level process flow and key performance targets: full assessment pathway for multisource products – facilitated by access to maturity level 3 or 4 national medicines regulatory authority or regional regulatory system reports



Notes:

KPI: key performance indicator.

NRA: national regulatory authority

^a Requirements are described in a separate guideline.

^b All WHO and stop clock times are in calendar days.

^c If at the end of the second-round assessment, the WHO assessment team cannot reach a positive assessment decision or the inspection team, following a desk review, requires an on-site inspection of the manufacturing site or the contract research organization, the application will no longer be considered as submitted via the facilitated full assessment procedure. Assessment may however continue via the regular full assessment and inspections procedure.

Appendix 4. Characteristics of the prequalified pharmaceutical product to be made available for public access on the WHO website

- WHO product reference number
- International nonproprietary name of active pharmaceutical ingredient
- dosage form and strength
- trade name of the product (if applicable)
- name of applicant and official address
- name of manufacturer of finished pharmaceutical product
- physical address of manufacturing site (and unit, if applicable)
- name of active pharmaceutical ingredient manufacturer, physical address of manufacturing site (and unit, if applicable)
- product description, as in finished pharmaceutical product specifications (for example, coated, scored, or other characteristics)
- pack size, primary and secondary packaging materials
- storage conditions
- shelf-life (provisional, if applicable)
- summary of product characteristics
- package leaflet
- labelling